SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Relugolix: daily hormone treatment

Advanced hormone-sensitive prostate cancer · NCCP 00830 · version 3 · 28-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
This is androgen deprivation therapy (ADT): it lowers testosterone to help control prostate cancer. With radiotherapy, the aim may be cure; in advanced disease, it is part of ongoing cancer control.

Schedule
Relugolix 360 mg on the first day, then 120 mg once daily without planned breaks. Your specialist sets the duration and any prescribed changes.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Relugolix
GNRH RECEPTOR ANTAGONIST

- ☐ Hot flushes, sweating, fatigue and changes in sleep or mood
- ☐ Reduced sex drive, erection difficulties and reduced fertility
- ☐ Loss of bone strength, fracture risk, muscle loss and changes in weight
- ☐ Changes in glucose, cholesterol and cardiovascular risk
- ☐ Heart-rhythm changes; review other medicines that affect the QT interval
- ☐ Diarrhoea, constipation, nausea and changes in liver blood tests
- ☐ Allergic reactions; important interactions with other medicines

Discuss treatment aim, duration, benefits and uncertainty, alternatives, sexual health and fertility, hot flushes, fatigue, mood, bone and muscle effects, metabolic/cardiovascular risks, interactions, formulation-specific risks, tests and urgent contacts. If treatment is interrupted for more than 7 days, contact the team before restarting: a new loading dose is normally needed. Some interacting medicines require a different dose or separation by at least 6 hours. Do not change the dose yourself; ask pharmacy to check every new medicine.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00830 v3.

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